

Executive Summary

Project: AI-Enabled Community Resource Navigation & Analytics Platform

Status: Pilot-tested, award-recognized, institutionally funded prototype

Current Assets - Deployed Streamlit-based application - ~150 real-world uses across ~40 ZIP codes - Active pilot within Great Lakes Health System - Presented at DNV National Conference - Grant-funded with University at Buffalo involvement - Logged usage, ZIP-level demand, and category-level data

Core Reframe

This project is **not a consumer product** and not a growth startup. It is **institutional infrastructure** designed to surface community resource gaps and referral friction using real usage data. Its value is administrative and strategic, not user-facing scale.

Primary End Game

UB-facilitated internal deployment and/or licensing via tech transfer (Option 1), with reporting and analytics (Option 3) emerging downstream from real-world use.

What “Success” Means - IP ownership and rights clarified via Invention Disclosure - At least one institution (UB or partner health system) able to legally deploy, reference, or report on the platform - One tangible output: internal report, grant justification, or modest support/maintenance agreement - Project mentally and operationally closed unless external pull justifies continuation

This is a **low-effort, option-preserving strategy**, not an attempt to build a company.

2026 Directional Plan (Q1–Q4)

Q1 2026 — Formalization & Containment

Goal: Eliminate ambiguity and prevent further sunk-cost creep.

Actions - Submit Invention Disclosure Form (IDF) to UB Tech Transfer - Frame as: grant-funded software infrastructure for dynamic community resource referral and analytics - Clarify: - inventor status - UB's claim (if any) - internal-use vs. licensing posture - Freeze feature development (no UI polish, no growth work)

Deliverable - IDF reviewed and classified (ideally Option B: support licensing / internal use)

Decision Gate - If UB declines → project parked with clarified ownership - If UB supports → proceed to Q2

Q2 2026 — Reporting Assetization

Goal: Convert existing pilot data into an institutionally legible asset.

Actions - Produce one internal-facing report using existing data: - usage volume by category - ZIP-level demand distribution - categories with high failure or low availability - qualitative feedback summary (if available) - Keep analysis conservative and descriptive (no causal claims)

Audience - UB administrators - Great Lakes Health System leadership - Grant stakeholders

Deliverable - PDF or slide-based “Pilot Insights Report”

Decision Gate - If report generates interest → Q3 - If acknowledged but dormant → idle or stop

Q3 2026 — Institutional Pull Test

Goal: Determine whether there is real downstream demand.

Actions (pull-driven only) - Respond to requests for: - updated reports - extended pilot use - internal deployment - UB handles contracts and licensing language - Your role limited to maintenance and light reporting

Possible Outcomes - Small internal support/maintenance agreement - Grant-linked continuation - Formal internal adoption without cash (still valid)

Deliverable - Either a signed agreement or a clear signal of no further interest

Q4 2026 — Resolution

Goal: Close the loop decisively.

Actions - If value materialized: - document outcome - archive project as institutional infrastructure with deployment history - If no value materialized: - formally freeze project - retain documentation and provenance - move on cleanly

End-of-Year Success Criteria - No unresolved ownership questions - No ongoing maintenance obligation - Either modest institutional value extracted or a clean, rational exit

Bottom Line

This project will not become a startup or generate meaningful independent revenue. Its legitimate value lies in institutional use, reporting, and option preservation. The 2026 plan is designed to extract that value with minimal additional effort and then stop.